



Service Planning & Budgeting

Week 3 - Budgeting in Service





Learning Outcomes

The lesson aims to develop learners understanding on the process by which budgets are prepared, negotiated, monitored and acted upon.

Objectives

At the end of the session, learners should be able to;

- Understand joint planning
- Understand how to prepare and set budget
- Describe budgets and different types of budgets
- Understand budget management behaviour



Recap of Last Session

- What is Plan Format?
- What are the components of a good quality plan?
- Describe how planning techniques can assist in analysis, direction setting and action?



Joint Commissioning/ Joint Planning

Joint commissioning can be seen as the logical outcome of current requirements by the central government for collaborative activity around:

- Hospital discharge
- Continuing care
- Community care planning
- Assessment of individuals' needs
- Nursing home admissions



Why Joint Commissioning or Planning

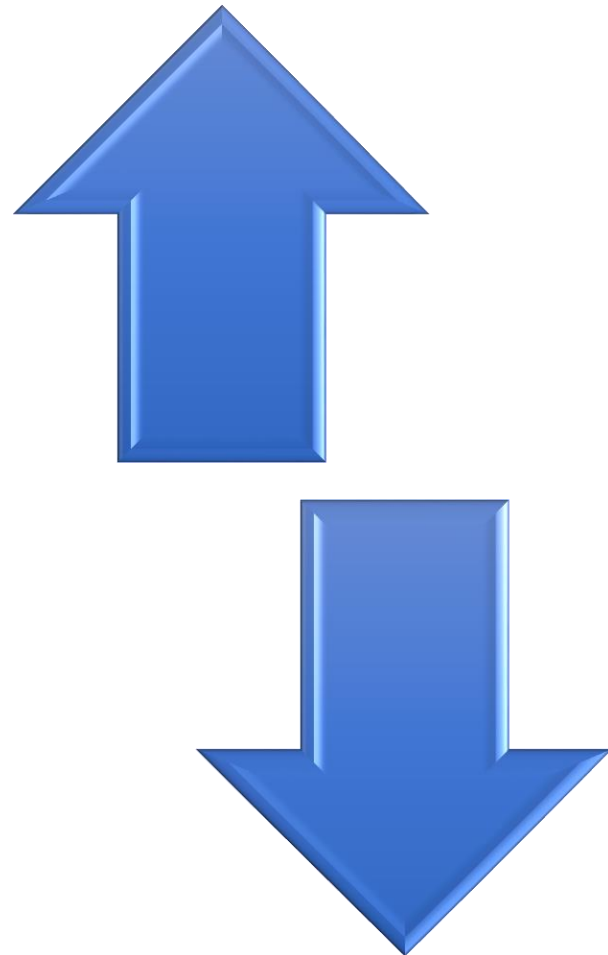
- Joint commissioning help agencies tackle the grey areas at the health and social care boundary.
- Joint commissioning fosters a focus on needs rather than organisations
- It also raises the profile of users and carers with the aim of improving access to services.



Joint Commissioning: What is it?

Joint commissioning is seen as the activity with agencies focusing on:

- Pooling information
- Combining expertise
- Agreeing main programme activities
- Taking decisions collaboratively on resource development
- Acting jointly through the planning and purchasing of services.





Budget Management and Preparation

Budget management is the total process by which budgets are prepared, negotiated, monitored and acted upon.



The concept of Budget in Health care

An expenditure budget is defined as:

The estimated cost of acquiring the use of the physical resources needed to pursue the actions planned to achieve the objectives, and ultimately the mission of the organisation.

And an income budget is defined as:

The estimated fees, charges, grants or other funding associated with the provision of one or more services, or activities.



Cont'd

Two aspects of the above definitions are important. Firstly, budgets reflect a chosen view of the future and it follows, therefore, that to some extent they will probably be incorrect. Secondly, there is an assumed link between the budget and the plan.



While budgets can be expressed in units of physical resource (e.g. staff hours, miles, etc.) they are more commonly expressed in financial terms and fall into two main types; those concerned with day to day running expenses and income (revenue), and others with the purchase of items broadly expected to last more than one year (capital).

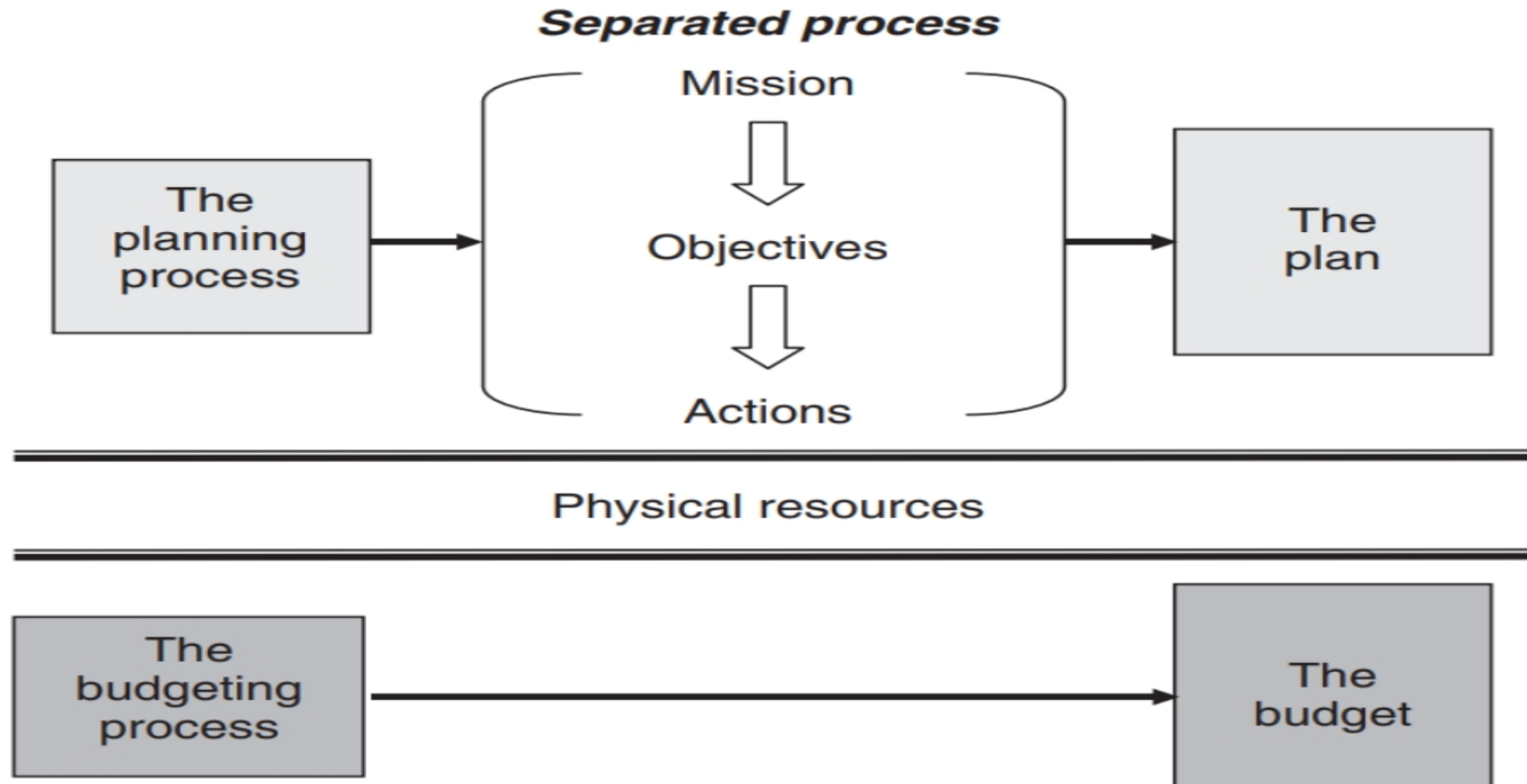


Fig.1: The separated process of budget
Source: Richard (2012)

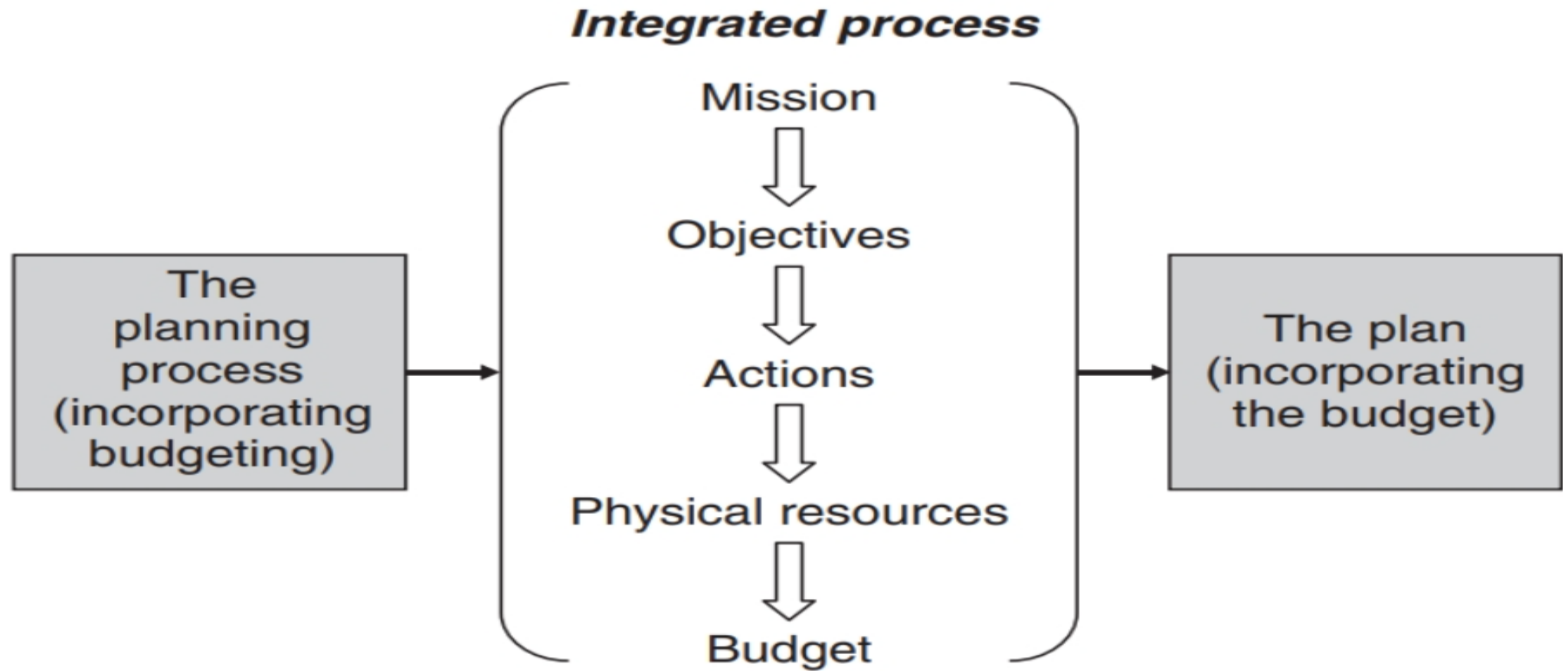


Fig. 2: The Integrated process of budget
Source: Richard (2012)



Budget management involves two major processes; preparation and control as shown in Figure 3, which also shows that these link with activity management which incorporates planning.

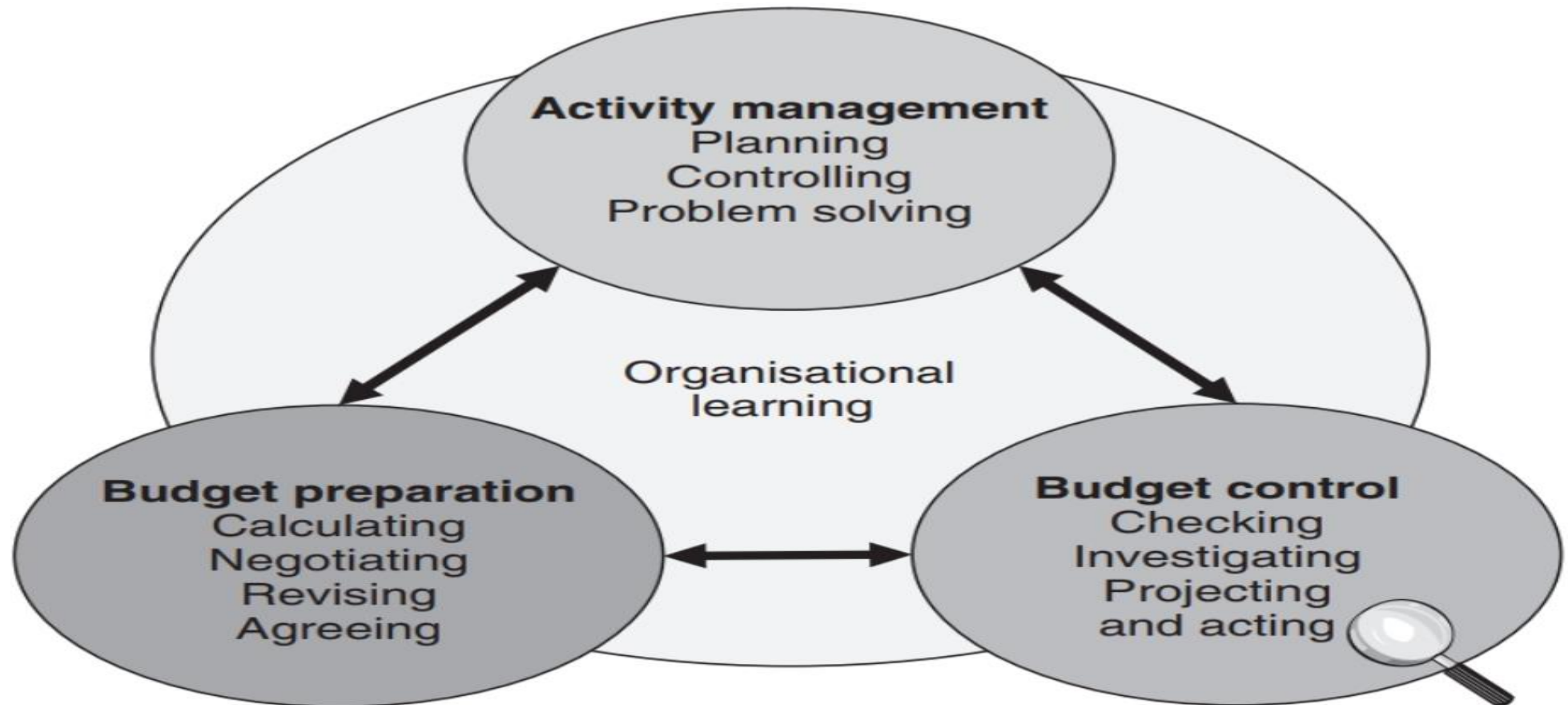


Fig. 3: Budget management in context
Source: Richard (2012)



The role of Budget in Managing the Organisation

An effective approach to budget management will:

- Stimulate and improve service planning, coordination and control across the organisation;
- Help ensure that plans are honest and realistic given the level of available resource, thereby minimising the risk that quality will be eroded;
- Encourage the discussion of service priorities.



Budget Preparation

The first of the four stages in budget preparation is calculation, which usually involves finance staff preparing an initial version of the budget based on the existing budget adjusted, or not, for the effects of inflation and other cost pressures between the years.

Budgets are financial targets and should be challenging yet realistic; qualities achieved through rigorous negotiation between the budget manager, their line manager and, ultimately, other stakeholders such as politicians and trustees, etc. To be effective in negotiation, the budget manager needs knowledge of the service, planned activity and the relationship this has with physical resources.

Either the plan and/or the budget may need revision as a result of negotiation, concluding with agreement as to the level of funding which should now be sufficient to resource the planned service.



Calculating Budget

There are two broad approaches to preparing a budget; rational and historic. Rational approaches attempt to relate financial budgets to physical resource requirements which, in turn, reflect the volume and quality of planned service. Rational approaches include; planned, policy-based budgeting, thematic, needs-led and zero-based, etc

While a good case can be made for rational budgeting, this is a time-consuming process that is likely to yield little benefit if done each year, particularly in respect of services that are relatively stable.

With historic, or incremental budgeting as it is often known, a future budget is based on previous actual figures or, more likely, the existing budget. In practice, most organisations use a mixture of rational and historic techniques, skewed towards the historic. While rational budgeting may be used for new services or as part of a fundamental review, historic budgeting will tend to be used elsewhere.



Irrespective of whether a rational or historic approach is adopted, all budgets comprise two elements:

- the volume of physical resource to be purchased (e.g. the number of staff to be employed); and
- the price (e.g. annual salary plus employer's national insurance and pension contributions).



Rational Budgeting: an approach to budgeting



There are several different forms of rational budgeting including plan-led and zero-based, which are outlined below.



Plan-led budgeting involves getting an early indication of the overall budget situation [i.e. the volume of physical resources required] which will allow those involved in planning and budgeting to be realistic about their aspirations



Zero-based budgeting: involves calculating budget by reference to what is needed rather than previous or existing budget levels. Potentially, zero-base budgeting is a creative process involving deep consideration of what is to be done, how and to what standard. Those involved in zero-based budgeting should be prepared to:

- challenge commonly held views regarding clients and their needs, the service and professional practice;
- develop a deep understanding of service outcomes and how these might be achieved;
- innovate in terms of service design and delivery.



Watch the videos below and discuss the key themes

<https://www.investopedia.com/terms/z/zbb.asp>

<https://www.youtube.com/watch?v=t0KLvsvShsk>



Reasons for lower and higher budgets arising from zero-based budgeting

Lower budgets may result from

Improved purchasing arrangements leading to economy gains.

More efficient use of resources resulting in less volume needing to be purchased.

Changing delivery processes which may affect the type, volume and mix of resources required. Overall the value of the resources required may fall.

A reduction in planned service volumes – therefore, less resource required.

Higher budgets may result from

A need to correct problems arising from years of pushing costs into the future.

The process revealing practices or standards that are illegal or unacceptable – additional resources may be required to remedy this situation.

Recognising that a service which might be economic and efficient is not actually meeting the needs of clients and, therefore, of low value – additional resources are required.

An increase in planned service volume or quality – therefore, more physical resources required.



Example zero-based budgeting questions

1. What is the reason for visiting the client?
2. Are assessments necessary?
3. Why is it necessary to visit the client in their own home?
4. Why not request that the client visits us or contacts us by telephone?
5. How often should each client be visited?
6. Could the assessment be undertaken as part of any other visit?

If a visit to the client at home is necessary

7. By what means should staff travel?

If, for example, cars are deemed necessary

8. How might journeys be planned in the most travel efficient way?
9. Which is the most cost effective way of organising car travel (e.g. pool cars, company cars, use of private cars)?

If private cars are to be used

10. What is the most appropriate rate to reimburse at?

We've updated our read aloud



Zero-based budgeting – advantages and limitations

Advantages are that it...

Involves challenging the current service.

Is potentially creative.

Engages managers and service professionals in budgeting.

Is likely to ensure recognition of the service user viewpoint.

Is a source of organisational learning and demonstrates a commitment to value for money.

Limitations are that it...

Is time consuming and can become unworkable.

Requires an ability and willingness to challenge at all organisational levels.

Places additional demands on finance staff.

Demands considerable non-financial information.

Can lead to an outcome that may conflict with the preferences of key stakeholders.



Historic Budgeting: an approach to budgeting

With historic budgeting, the budget for next year is based on previous actual expenditure or, more frequently, the budget relating to the current year.

In practice there are two forms of historic budgeting; full and partial. With the former version a fair attempt is made to ensure that the budget for next year adequately takes account of all financial implications of known or anticipated changes in the environment, the service offered, or the price of resources required. Table 4 is an example of how full incremental budgeting might look in respect of an establishment.



Table 4: Example of Historic budgeting

Establishment A	£
Budget for 2012/13 (current year)	1,450,000
Anticipated service changes (2013/14)	
Reduction in expected occupancy	–30,000
New quality standards	40,000
Implementation of new pay structure	20,000
Staff restructuring	–25,000
	1,455,000
Inflation (Estimated at 3% and rounded up)	44,000
Budget for 2013/14	1,499,000



Examples for Budgets

KANE COUNTY HUMAN RESOURCE SPECIAL SERVICE DISTRICT
dba KANE COUNTY HOSPITAL With Operated Nursing Facilities
BUDGET for the year ended 12/31/2020

Draft Final

ENTERPRISE FUND

	PRIOR YEAR	CURRENT YEAR	BUDGET
REVENUES			
Taxes: Property			
Other: Sales Tax	2,232,205	2,233,261	2,333,772
Fee-in-lieu of Taxes			
Charges for services	40,883,331	41,940,197	43,533,749
Interest Income	322,067	672,913	672,912
Other: donations/grants	251,005	280,841	280,836
Other Financing Sources:			
Transfers From Other Funds	0	0	0
Contribution from Fund Bal.			
TOTAL REVENUES	43,688,608	45,127,212	46,821,269
EXPENSES			
Salaries and Benefits	6,133,289	5,948,695	6,288,979
Other Operating Expenses	30,695,145	35,178,482	36,930,983
Depreciation	871,326	823,461	1,035,206
Capital Outlay			
Debt Service	60,273	189,662	525,166
Other:			
Other Financing Uses:			
Transfers To Other Funds			
Contribution to Fund Bal.			
TOTAL EXPENSES	37,760,033	42,140,300	44,780,334
INCOME OR (LOSS)	5,928,575	2,986,912	2,040,935

Health and Human Services (HHS) Grant FY 2013-2014 Budget Proposal

Organization Name: Sunny Beach Hospital

Title/Department: Director of Nursing

Contact Person: John Doe

Phone Number: 318-999-9999

Email Address: joe.doe@hospital.com

Fax Number: 318-999-9998

Reimbursable Limit: 10,000.00

Match Amount (10%): 1,000.00

TOTAL OBLIGATION AMOUNT: 11,000.00

SAMPLE

A. CASH EXPENDITURES

Cashability Planning Guide	Description	Amount
CPG 1 - Healthcare System Preparedness	Training	\$ 5,000.00
CPG 2 - Recovery	HVA Assessment, Development of SOP/COOP	\$ 750.00
CPG 3 - Emergency Operation Coordination	HEICB vests, Command Center Equipment	\$ 450.00
CPG 5 - Mass Fatality	Body Bags	\$ 300.00
CPG 6 - Information System	700 MHz Radio	\$ 1,000.00
CPG 10 - Medical Surge	Bed capacity equipment, Decontamination Tent	\$ 2,000.00
CPG 14 - Responder Safety	PPE, Pharmaceutical Cache	\$ 2,000.00
CPG 15 - Volunteer Management	"Just In Time" Training Modules	\$ 1,000.00
TOTAL CASH EXPENDITURES		\$ 12,600.00

B. NON CASH IN-KIND CONTRIBUTIONS

Staff Time	\$ -
Meeting Space	\$ 300.00
Storage/Rental Space	\$ -
Mileage Reimbursement	\$ -
TOTAL NON CASH IN-KIND CONTRIBUTIONS	\$ 300.00

C. TOTAL AMOUNT PROPOSED (Cash Expenditures + In-Kind Contributions)

TOTAL AMOUNT	\$ 12,900.00
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D. OVERAGE (Total Amount Proposed - Total Obligation Amount)

AMOUNT OVER TOTAL OBLIGATION	\$ 1,900.00
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By signing below, you are ensuring that HHS grant funds will be spent in accordance with the guidelines as specified in your Participation Agreement.

CEO's Name (Print):

Date:

CEO's Signature:



Indian River County Hospital District
Final Budget
Fiscal Year 2019 - 2020

	2016/2017 Approved Budget	2017/2018 Approved Budget	2018/2019 Approved Budget	2018/2019 Projected 09/30/19	2019/2020 Proposed Budget
Cash Balance Brought Forward	3,895,915	2,949,231	2,021,868	2,225,734	4,088,123
Less: Prior Year Reserves					
Improvement & Betterment of GHC	30,000	60,000	60,000	60,000	120,000
Improvement & Betterment of HSB	85,000	145,000	100,000	50,000	100,000
Contingency Reserve	1,500,000	1,500,000	1,500,000		1,500,000
Reserve Fund - Indigent care cont	196,421	265,000	718,280	100,000	2,457,989
Health Needs Assessment Reserve	250,000				
Partnership Process Reserve		1,000,000	350,000	205,000	
Total Prior Year Reserves	2,061,421	2,970,000	2,728,280	415,000	4,177,989
Net Cash Balance Brought Forward	1,834,494	(20,769)	(706,412)	1,810,734	(89,866)
<u>Revenues</u>					
Taxes: Millage Per \$1,000 = 0.8011					
Ad Valorem Tax	12,850,895	14,496,066	15,880,620	15,904,500	14,912,557
Interest Income	11,600	15,000	15,000	17,600	15,000
Rental Income - HSB	279,600	246,000	246,000	242,300	247,000
Total Revenues and Other Financing Sources	13,142,095	14,757,066	16,141,620	16,164,400	15,174,557
			(75,905)		
<u>Program Expenditures</u>					
<i>Indian River Medical Center</i>					
Indigent Care-Hospital	5,763,036	6,358,449	7,560,223	6,263,099	6,000,000
Indigent Care Variance					
We Care assistance		200,000			
Partners Program	1,325,000	1,325,000	1,682,520	1,682,520	1,682,520
Incentive	100,000	100,000			

<https://www.sample.net/budget/hospital-budgets/#Nursing-Services-Budget-Template>



The advantages and limitations associated with historic budgeting

Historic budgeting

Advantages are that it . . .

Is easy to undertake and relatively inexpensive, particularly in centralised organisations.

Is relatively quick to complete so budget preparation can start later in the year.

Results in relatively stable patterns of funding and service provision which assists central control.

Is less likely to result in resource rivalry due to increases or decreases tending to be applied incrementally and experienced to a similar extent by different services.

Limitations are that it . . .

Encourages the separation of planning and budgeting thereby breaking the link between activity and resources.

Encourages the perpetuation of existing services and ways of operating

Does not normally lead to any slack in the budget being identified.

Causes budget preparation to become a technical financial process from which many managers withdraw leading to disempowerment.

We've updated our read aloud
Give it a try here.



What Budget managers should carefully consider?

- **Budget Management** involves the coordination of financial and nonfinancial planning to satisfy organizational goals and objectives.
- No foolproof method exists for preparing an effective budget
- However, budget makers/ Managers should carefully consider the following conditions for the proper budgetary management for the success



1. Top management support

All management levels must be aware of the budget's importance and must know that the budget has top management's support. Top management, then, must clearly state long-range goals and broad objectives. These goals and objectives must be communicated throughout the organization.

2. Participation in goal setting

Management uses budgets to show how it intends to acquire and use resources to achieve long-range goals. Employees are more likely to strive toward organizational goals if they participate in setting them and in preparing budgets. Often, employees have significant information that could help in preparing a meaningful budget. Also, employees may be motivated to perform their own functions within budget constraints if they are committed to achieving organizational goals.



3. Communicating results

People should be promptly and clearly informed of their progress. Effective communication implies (1) timeliness, (2) reasonable accuracy, and (3) improved understanding. Managers should effectively communicate results so employees can make any necessary adjustments in their performance.

4. Flexibility

If significant basic assumptions underlying the budget change during the year, the planned operating budget should be restated. For control purposes, after the actual level of operations is known, the actual revenues and expenses can be compared to expected performance at that level of operations



5. Follow-up

Budget follow-up and data feedback are part of the control aspect of budgetary control. Since the budgets are dealing with projections and estimates for future operating results and financial positions, managers must continuously check their budgets and correct them if necessary. Often management uses performance reports as a follow-up tool to compare actual results with budgeted results.



Summary

- <https://courses.lumenlearning.com/sac-managacct/chapter/introduction-to-budgeting-and-budgeting-processes/>
- <https://www.investopedia.com/terms/b/budget.asp>



References

- Department of Health (2008). Joint Strategic Needs Assessment Quality Assurance Toolkit NHS East of England
- Richard, F. (2012). *Planning and budgeting skills for health and social work managers*. London: Sage.
- Reed, Z. (n.d). *Health and social care: what future for joint planning?* Camden Social Services Department. Available at: <http://www.psi.org.uk/publications/archivepdfs/Making/18-REED.pdf> [Accessed 05 April 2020].



Assignment -1

- Assignment 1(Case study: planning and budgeting) 2000 +/- 10%

Total Weighting: 50%

Intended Learning Outcomes: 1 and 2

ILOs

1. Use appropriate techniques to prepare and lead a planning process to create service direction.
2. Prepare and negotiate service budgets in single and multi-agency contexts.

Let's discuss the Assignment Structure-

Formative feedback deadline: